

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT				1. CONTRACT ID CODE		PAGE 1 OF 5 PAGES			
2. AMENDMENT/MODIFICATION NUMBER 0001			3. EFFECTIVE DATE 05/08/2025		4. REQUISITION/PURCHASE REQUISITION NUMBER 15B50525PR000343		5. PROJECT NUMBER (If applicable)		
6. ISSUED BY Federal Bureau of Prisons Field Acquisition Office U.S. Armed Forces Reserve Complex 346 Marine Forces Drive Grand Prairie, TX 75051			CODE 15BCMS Hannah Mendenhall (O) 202-913-1667 hmendenhall@bop.gov		7. ADMINISTERED BY (If other than Item 6) Federal Bureau of Prisons FMC Carswell J Street Building 3000 Fort Worth, TX 76127		CODE 15B505		
8. NAME AND ADDRESS OF CONTRACTOR (Number, street, country, state and ZIP Code)						(X)		9A. AMENDMENT OF SOLICITATION NUMBER	
						X		15BFA026R00000004	
								9B. DATED (SEE ITEM 11)	
								04/06/2026	
								10A. MODIFICATION OF CONTRACT/ORDER NUMBER	
								10B. DATED (SEE ITEM 13)	
CODE			FACILITY CODE						

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☒ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers ☒ is extended, ☐ is not extended.

Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods: (a) By completing items 8 and 15, and returning 1 copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or electronic communication which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEDGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by letter or electronic communication, provided each letter or electronic communication makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA (If required)

**13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS.
IT MODIFIES THE CONTRACT/ORDER NUMBER AS DESCRIBED IN ITEM 14.**

CHECK ONE	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: (Specify authority) THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NUMBER IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES (such as changes in paying office, appropriation date, etc.) SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
	D. OTHER (Specify type of modification and authority)

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return 1 copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION (Organized by UCF section headings, including solicitation/contract subject matter where feasible.)

Amendment 0001 is hereby issued to add H.15 - Transition of Services and Continuity of Care, H.16 - Quality Assurance Plan (QAP), Attachment 11- Quality Assurance Plan, and 52.222 - 90 - Addressing DEI Discrimination by Federal Contractors (APR 2026).

All other Terms, Conditions, and Line Items remain the same.

THIS AMENDMENT EXTENDS THE SOLICITATION CLOSING DATE/TIME TO: 06/12/2026 - 14:00 CT US/Central

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER (Type or print)		16A. NAME AND TITLE OF CONTRACTING OFFICER (Type or print) Hannah M Mendenhall Contracting Officer	
15B. CONTRACTOR/OFFEROR (Signature of person authorized to sign)	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA By (Signature of Contracting Officer)	16C. DATE SIGNED

Previous edition unusable

Table of Contents

<u>Section</u>	<u>Description</u>	<u>Page Number</u>
A	Solicitation/Contract Form.....	1
B	Supplies or Services and Prices/Costs.....	3
C	Description/Specifications/Statement of Work.....	3
D	Packaging and Marking.....	3
E	Inspection and Acceptance.....	3
F	Deliveries and Performance.....	3
G	Contract Administration Data.....	3
H	Special Contract Requirements.....	3
	H.15 Transition of Services and Continuity of Care.....	3
	H.16 Quality Assurance Plan (QAP).....	4
I	Contract Clauses.....	4
	52.222-90 Addressing DEI Discrimination by Federal Contractors (APR 2026).....	4
J	List of Attachments.....	5
K	Representations, Certifications and Other Statements of Offerors.....	5
L	Instructions, Conditions and Notices to Offerors.....	5
M	Evaluation Factors for Award.....	5

Section B - Supplies or Services and Prices/Costs

CMCS-FMC CARSWELL - COMPREHENSIVE MEDICAL

SCHEDULE OF SUPPLIES/SERVICESCONTINUATION SHEET

Section C - Description/Specifications/Statement of Work**No Clauses****Section D - Packaging and Marking****No Clauses****Section E - Inspection and Acceptance****No Clauses****Section F - Deliveries and Performance****No Clauses****Section G - Contract Administration Data****No Clauses****Section H - Special Contract Requirements****Clauses By Full Text**

Clause H.15 Transition of Services and Continuity of Care was added or modified.

H.15 Transition of Services and Continuity of Care

(a) General Requirement.

The Contractor shall use best efforts to support an orderly, efficient, and seamless transition of medical services at the expiration or termination of this contract. The contractor is expected to work collaboratively with the Government and any successor contractor to facilitate continuity of care and minimize disruption to patient services. These transition

support requirements remain in effect for a period of 60 days following contract expiration or termination, unless otherwise shortened in writing by the contracting officer.

(b) Coordination with Successor Contractor.

The Contractor shall coordinate with the incoming contractor to transfer all relevant administrative, scheduling, and operational information necessary to continue medical services without interruption. This includes, but is not limited to, scheduled patient appointments, provider assignments, and related care coordination activities.

(c) Scheduled Appointments Beyond Contract Period.

The Contractor shall not cancel, reschedule, or otherwise disrupt patient appointments that are scheduled to occur beyond the contract expiration date without prior written approval of the Contracting Officer. To the maximum extent practicable, the Contractor shall work with the successor contractor to transfer such appointments, provided that the transfer is feasible and both contractors operate within the same or a compatible network of medical providers.

(d) Continuation of Services.

If the transfer of scheduled appointments to the successor contractor is not feasible, the Government reserves the right to require the Contractor to continue performance for those appointments. Such continued performance may be authorized through a purchase order or other appropriate contractual instrument. Services performed under this paragraph shall be provided in accordance with the rates, terms, and conditions of the most recent exercised option of this contract. The Contractor shall support and comply with such authorization to ensure uninterrupted medical care.

(e) Continuity of Care Priority.

All transition and phase-out activities shall be conducted in a manner that prioritizes patient safety, continuity of care, and compliance with all applicable laws, regulations, and standards of medical practice.

(f) No Additional Cost for Transition Cooperation.

The Contractor's obligation to cooperate in the transition and transfer of services as described in this section shall be performed at no additional cost to the Government, except as otherwise authorized in writing by the Contracting Officer.

Clause H.16 Quality Assurance Plan (QAP) was added or modified.

H.16 Quality Assurance Plan (QAP)

The contractor shall adhere to and comply with the QAP included in this solicitation (attachment 11 – Quality Assurance Plan), including all performance monitoring, documentation, and reporting requirements contained therein.

Section I - Contract Clauses

Clauses By Reference

52.252-2 CLAUSES INCORPORATED BY REFERENCE (FEB 1998)

This contract incorporates one or more clauses by reference, with the same force and effect as if they were given in full text. Upon request, the Contracting Officer will make their full text available. Also, the full text of a clause may be accessed electronically at this/these address(es): www.acquisition.gov

Clause	Title	Fill-ins (if applicable)
52.222-90	Addressing DEI Discrimination by Federal Contractors (APR 2026)	

Section J - List of Attachments**No Clauses**

Identifier	Title	Number of Pages
12	Attachment 11-Quality Assurance Plan.pdf - ADDED	5

Section K - Representations, Certifications and Other Statements of Offerors**No Clauses****Section L - Instructions, Conditions and Notices to Offerors****No Clauses****Section M - Evaluation Factors for Award****No Clauses**

1 INTRODUCTION

This Quality Assurance Plan (QAP) aligns with the requirements set forth in the Performance Work Statement (PWS) titled Comprehensive Medical Services – FMC Carswell. It establishes a nationwide strategy for the procurement and oversight of all medical service contracts. The plan addresses a comprehensive solution to previously identified deficiencies, incorporating measurable performance standards aimed at achieving the desired outcomes related to quality and timeliness of care. It also includes standards for maintaining documentation pertinent to the performance ratings outlined in the QAP.

The plan defines the procedures and guidelines the Federal Bureau of Prisons (BOP) will follow to ensure the contractor meets the required performance standards and service levels. Additionally, it assigns responsibilities to BOP staff for conducting performance surveillance, completing performance reporting tasks, and ensuring the proper documentation is maintained to support the contractor's performance ratings.

1.1 Purpose

- 1.1.1 The purpose of the QAP is to describe the systematic methods used to monitor performance and identify required documentation and resources to be employed. The QAP provides a means for evaluating whether the contractor is meeting the performance standards/quality levels identified in the PWS and the contractor's quality control plan (QCP), and ensures the government pays only for the level of services received.
- 1.1.2 This QAP defines the roles and responsibilities of all contract administration personnel, identifies the performance objectives, defines the methodologies used to monitor and evaluate the contractor's performance, describes quality assurance documentation requirements, and describes the analysis of quality assurance monitoring results.

1.2 Performance Management Approach

- 1.2.1 The PWS structures the acquisition around *what* service or quality level is required, as opposed to *how* the contractor should perform the work (i.e., results, not compliance). This QAP will define the performance management approach taken by the BOP to monitor and manage the contractor's performance to ensure the expected outcomes or performance objectives communicated in the PWS are achieved. Performance management rests on developing a capability to review and analyze information generated through performance assessment. The ability to make decisions based on the analysis of performance data is the cornerstone of performance management; this analysis yields information that indicates whether expected outcomes for the project are being achieved by the contractor.

- 1.2.2 Performance management represents a significant shift from the more traditional quality assurance (QA) concepts in several ways. Performance management focuses on assessing whether outcomes are being achieved and to what extent. This approach migrates away from scrutiny of compliance with the processes and practices used to achieve the outcome. A performance-based approach enables the contractor to play a large role in how the work is performed, if the proposed processes are within the stated constraints. The only exceptions to process reviews are those required by law (federal, state, and local) and compelling business situations, such as safety and health. A “results” focus provides the contractor flexibility to continuously improve and innovate over the course of the contract if the critical outcomes expected are being achieved and/or the desired performance levels are being met.

1.3 Performance Management Strategy

- 1.3.1 The contractor is responsible for the quality of all work performed. The contractor measures that quality through the contractor’s quality control (QC) plan. QC is work output, not workers, and therefore includes all work performed under this contract regardless of whether the work is performed by contractor employees or subcontractors. The contractor’s QCP will set forth the staffing and procedures for self-inspecting the quality, timeliness, responsiveness, customer satisfaction, and other performance requirements in the PWS. The contractor will develop and implement a performance management system with processes to assess and report its performance to the designated government representative. The contractor’s QCP will also set forth the staffing and procedures for self-inspecting the quality, timeliness, responsiveness, customer satisfaction, and other performance requirements in the PWS. This QAP enables the government to take advantage of the contractor’s QCP.

2 ROLES AND RESPONSIBILITIES

2.1 The Contracting Officer (CO)

The CO is responsible for monitoring contract compliance, contract administration, and cost control, and resolving any differences between observations documented by the contracting officer's representative (COR) and the contractor.

2.2 The COR

The COR is designated in writing by the CO to act as their authorized representative to assist in administering a contract. COR limitations are contained in the written appointment letter. The COR is responsible for technical administration of the contract and ensures proper technical compliance of the contractor’s performance. The COR is not empowered to make any contractual commitments or authorize any contractual changes on the government’s behalf. Any changes the contractor deems may affect contract price, terms, or conditions must be referred to the CO for action.

3 IDENTIFICATION OF REQUIRED PERFORMANCE STANDARDS

The QAP-related performance standards are included in Attachment 1: Performance Requirements Summary.

4 METHODOLOGIES TO MONITOR PERFORMANCE

4.1 Monitoring Techniques

To minimize the performance management burden, simplified monitoring methods shall be used by the government to evaluate contractor performance when appropriate. The primary methods of monitoring are:

- *DIRECT OBSERVATION*
- *BOP DASHBOARD*
- *PERIODIC SAMPLING*
- *REVIEW OF DOCUMENTS*

5 QUALITY ASSURANCE DOCUMENTATION

5.2 Monitoring Forms

The government's QAP monitoring, accomplished by the COR, will be reported using the monitoring form in Attachment 2. The forms, when completed, will document the government's assessment of the contractor's performance under the contract to ensure the required results are being achieved.

5.2.1 The COR will retain a copy of all completed QAP monitoring forms in a file. Upon contract completion, this file will be transferred to the contracting officer.

6 ANALYSIS OF QUALITY ASSURANCE ASSESSMENT

6.1 Determining Performance

6.1.1 The government shall use the monitoring methods cited to determine whether the performance standards have been met. If the contractor has not met the requirements, they may be asked to develop a corrective action plan to show how and by what date they intend to bring performance up to the required levels.

6.2 Reviews and Resolution

6.2.1 The CO and/or COR may require the contractor to meet with the BOP to discuss the performance of the contract. Items discussed may include:

- Monthly performance assessment data
- Issues and concerns of both parties
- Projected services
- Any corrective action
- Recommendations for improved efficiency and/or effectiveness

6.3.2 The CO and COR must coordinate and communicate with the contractor to resolve issues and concerns regarding marginal or unacceptable performance.

ATTACHMENT 1: PERFORMANCE REQUIREMENTS SUMMARY

Required Task	Performance Standard	Acceptable Quality Level	Method to Monitor Performance
Timeliness of scheduling outside/inside medical care	100%	90%	Direct observation, BOP Dashboard, periodic sampling, review of documents.
Urgent Medical/Dental appointments are scheduled within 14 days or sooner if clinically indicated	100%	90%	Direct observation, BOP Dashboard, periodic sampling, review of documents
Hospital utilized maintains a quality accreditation (i.e. JC, NCCHC, etc.)	100%	100%	Periodic sampling, review of documents.
Provide inpatient and outpatient facility services including professional services which conform to community standards and all local, state and Federal laws and regulations applicable to the delivery of health care to members of the general public.	100%	95%	Periodic sampling, review of documents.

ATTACHMENT 2: SAMPLE QUALITY ASSURANCE MONITORING FORM

SERVICE or STANDARD: Hospital utilized maintains quality accreditation (i.e. JC, NCCHC, etc.)

SURVEY PERIOD: _____

MONITORING METHOD (check):

☐ Direct Observation ☐ BOP Dashboard ☐ Periodic Sampling ☐ Review of Documents

LEVEL OF MONITORING (check):

☐ Monthly ☐ Quarterly ☐ As needed

PERCENTAGE OF ITEMS SAMPLED DURING SURVEY PERIOD: 100 %

ANALYSIS OF RESULTS:

Observed Service Provider Performance Measurement Rate: 100%

Service Provider's Performance (check):

☐ Meets Standards

☐ Does Not Meet Standards

Narrative of Performance During Survey Period: _____

PREPARED BY: _____ **DATE:** _____